National Cancer Control Programme (NCCP) 00339 · clinician draft

Vinorelbine and cisplatin · 21-day cycle

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Cisplatin-containing day: 1

Active medicines: vinorelbine + cisplatin

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg orally Day 1; 80 mg orally Days 2–3	Before systemic anticancer therapy (SACT) on Day 1, then daily on Days 2–3 Aprepitant inhibits CYP3A4 and can increase vinca toxicity; prescriber/pharmacy must review the combination and monitor.
Ondansetron	16 mg orally	Day 1 Adjust for QT/individual risk.
Antiemetic dexamethasone reference	12 mg orally Day 1; 8 mg orally daily Days 2–4	General high-risk NCIS V8 row Reconcile the antiemetic course and interactions into one prescription; this is a reference course requiring local confirmation.
Olanzapine	5 mg orally once daily	Day 1 and Days 2–4 General high-risk NCIS reference; review sedation, falls, metabolic risk and interaction with metoclopramide. Consultant may individualise.
Metoclopramide	10 mg orally three times daily as prescribed	Subsequent days NCIS row does not specify an end day or PRN qualifier; prescriber/pharmacy must specify duration and suitability. Co-prescribing with olanzapine increases extrapyramidal risk; assess neurological contraindications and interactions.

Additional phase-specific support

- Follow your prescribed anti-sickness and constipation-prevention plan. Cisplatin days need kidney-protecting fluids and electrolyte checks. Do not repeat the full cisplatin anti-sickness course on a vinorelbine-only day unless prescribed.
- Cisplatin pre/post hydration and electrolyte prescription per local policy; monitor fluid balance.
- Prophylactic bowel plan; assess CYP3A interactions, including aprepitant.
- This reference is not a prescription; doses and duration require local oncology/pharmacy confirmation.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00339 v7 · pNCCP 00339 pp1–4; NCIS V8 Table 1 · NCCP NCIS antiemetics V8

Sources checked 2026-11-02 · SACTCheck v0.84.28 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Vinorelbine-only day: 8/15

Active medicines: vinorelbine

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- No routine antiemetic prophylaxis for minimal risk; individualise rescue medicines.
- Continue the prescribed constipation plan. Do not restart the full cisplatin antiemetic course automatically.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

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